Metro Health Medical Center - Acute Care Pavilion

CLINICAL DOCUMENTATION — NARRATIVE SPECIALTY CONSULT NOTE V1

Patient: Daniel Harris | **MRN:** PAT-60018 | **DOB:** 1961-02-03 | **Gender:** Male | **Doc ID:** NARRATIVE_0018
Encounter Date: 2025-06-17 | **Discharge/Note Date:** 2025-06-19 | **Provider:** Dr. Marcus Vance, DO

REASON FOR CONSULTATION

Consult requested by primary medical team regarding refractory clinical optimization and therapeutic titration. Consultant team: Cardiovascular Medicine Consult Service.

CHRONOLOGICAL CLINICAL NARRATIVE

CHRONOLOGICAL CLINICAL NARRATIVE: We were asked to evaluate patient Daniel Harris, who was admitted three days ago with complex multisystem decompensation. On initial presentation, initial biomarker profiling revealed calcium of 9.46 mg/dL. Subsequent evaluation demonstrated that serum white blood cell count was 2.8 10*3/uL (LOW). Prior outpatient regimens had proven insufficient to achieve therapeutic stability.

HOSPITAL COURSE & TRAJECTORY

HOSPITAL COURSE & TRAJECTORY: During inpatient specialty evaluation, the consultant team adjusted pharmacological therapy. The patient was initiated on Amlodipine 2.5 mg PO every morning. Following this intervention, trajectory assessment showed total cholesterol of 167.7 mg/dL. Clinical response has been favorable with marked improvement in objective metabolic parameters.

FOCUSED PHARMACOTHERAPY RECOMMENDATIONS

FOCUSED PHARMACOTHERAPY RECOMMENDATIONS: The specialty team recommends the following maintenance regimen: continue Metoprolol 25 mg PO twice daily; and titrate Amoxicillin 500 mg by mouth three times daily for 7 days. Avoid NSAIDs and nephrotoxic exposures.

CONSULTANT ASSESSMENT & SPECIALTY PLAN

The patient is stabilized and appropriate for transfer to the regular step-down floor. Re-evaluate laboratory markers in 48 hours. Thank you for this interesting consultation.